

Improve Your Pain Management Using a Quality Improvement Approach

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Objectives

- Understand FOCUS-PDSA quality improvement process
- Apply these concepts to improvement in pain management
- Identify facility opportunities for pain management improvement
- Develop pain management action plan using FOCUS-PDSA concepts

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Why Focus on Pain?

- Pain is the symptom most expected and most feared by dying patients
- Unrelieved pain can have enormous physiological and psychological effects on patients and their loved ones
- Pain negatively affects the quality of life by impairing daily functions, social relationships, sleep and/or self worth

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Pain Effects in the Elderly

- Impairs mobility: slows rehab
- Decreases socialization
- Increases depression and sleep disturbances
- Contributes to cognitive impairment
- Contributes to inadequate food intake and malnutrition
- Increases morbidity

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Common Types of Pain in the Elderly

- **Acute:** surgical, medical, musculoskeletal
- **Chronic:** surgical, medical arthritic, neuropathic, musculoskeletal
- **Cancer:** breast, prostate, colon, lung, acute and chronic

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Pain Quality Measures

- Short stay “post-acute” population
 - Percentage of residents with moderate or excruciating pain on the 14-day prospective payment system (PPS) Minimum Data Set (MDS) assessments
- Long “chronic” stay population
 - Percentage of residents with moderate or excruciating pain on non-PPS and non-admission assessments

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Where Do We Start?



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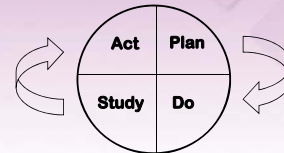
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Model for Improvement

What are we trying to accomplish?

How do we know that a change is an improvement?

What changes can we make that result in an improvement?



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FOCUS – PDSA

Find a process to improve

Organize a team that knows the process

Clarify current knowledge of the process

Understand causes of process variation

Select the process improvement

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FOCUS – PDSA

Find a process to improve

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Aim Statement

Mission Hills Nursing Center will implement an organizational approach to caring for all residents with pain that incorporates basic principles of pain assessment and treatment into patterns of daily practice

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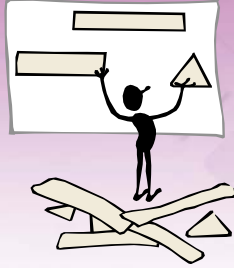
Organize a Team that Knows the Process



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Clarify Current Knowledge of the Process



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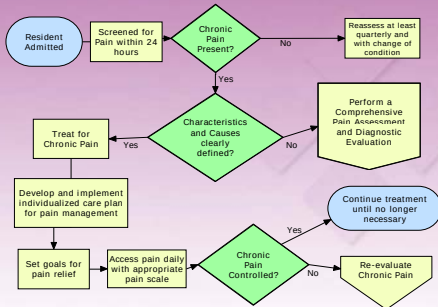
Key Steps to Effective Pain Management

- Screen residents routinely for pain
- Complete assessment for residents found to have pain on screening
- Identify resident's goal for pain management
- Address pharmacologic and non-pharmacologic treatments in plan of care
- Monitor effectiveness of treatments in meeting resident's goal
- Revise plan of care based on monitoring

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Pain Management Flow Chart



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Screen Residents

- Consider pain as the fifth vital sign
- Use a pain scale for screening
- Identify a pain scale for verbal, cognitively intact residents
- Identify a pain scale for non-verbal and/or cognitively impaired residents

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Standardized Screening Tools

Wong-Baker Faces Pain Rating Scale



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Standardized Screening Tools

Checklist of Nonverbal Pain Indicators (CNPI)		
Indicators:	With Movement	At Rest
Vocal Complaints - (nonverbal expression of pain demonstrated by moans, groans, grunts, cries, gasps, sighs)		
Facial Grimaces & Wincing - (furrowed brow, narrowed eyes, tightened lips, dropped jaw, clenched teeth, distorted expression)		
Bracing - (clutching or holding onto bed/chair, caregiver, or affected area during movement)		
Restlessness - (constant or intermittent shifting of position, rocking, intermittent hand motions, inability to keep still)		
Rubbing - (massaging affected area)		
Vocal Complaints - (verbal expression of pain using words, e.g., "ouch" or "that hurts"; cursing during movement, or exclamation of protest, e.g., "stop" or "that's enough")		

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Key Actions for Improvement in Screening Residents

- Process defined and understood by staff
- Standardized pain screening tools are used
- Responsibility for screening designated
- Process for “next step” when pain identified is defined and is understood by staff

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Complete Pain Assessment

- If pain is identified upon screening, need comprehensive pain assessment
- Elements of complete assessment
 - Description of pain
 - Location, intensity, frequency
 - Pain at its least/worst
 - Aggravating/alleviating factors
 - Current treatment
 - Response to current treatment

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Key Actions for Improvement in Comprehensive Assessment

- Revise assessment tool to ensure it has all recommended elements
- Protocol to complete assessment tool for residents who screen as having pain
- Check that assessment tool is being completed
- Use information in plan of care

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Care Planning

- Resident identified pain management goal
- Pharmacologic component
- Non-pharmacologic component
- Monitoring component

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Pharmacologic Component of Effective Treatment

- World Health Organization (WHO) Pain Ladder
- Pain algorithms
 - Mild pain
 - Moderate pain
 - Severe pain

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Critical Concepts in Managing Pain

- Administer medications routinely, not PRN
- Use the least invasive route of administration first
- Begin with low dose – titrate up
- Monitor and document effectiveness of medication daily
- Re-assess and adjust dose frequently to optimize pain relief while monitoring and managing side effects

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Key Steps for Improvement in Care Planning and Treatment

- Assessment data incorporated into care plan
- Responsibility for care plan development designated
- Care plan includes pharmacological and non-pharmacological interventions
- Staff, resident, and family education about effective approaches to pain management

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Monitoring Treatment Effectiveness

- Pain scales
 - Use appropriate tool considering physical and cognitive status of resident
 - Using same pain scale standardizes assessment across shifts, multiple staff, less subjective
- When
 - Minimally daily
 - With medication changes
 - With change in condition

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Key Steps in Monitoring

- Residents monitored for pain at least daily
- Pain scale used
- Responsibility for monitoring designated
- Results of monitoring recorded in medical record
- Change plan of care based on monitoring results

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Reassessment of Pain

- When
 - At regular intervals
 - With any complaints of increasing pain
- Elements of reassessment
 - Includes all elements of comprehensive assessment
- Who
 - Anyone requesting increasing frequency of PRN medication

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Key Steps in Reassessment

- Reassessment should include all components in comprehensive assessment
- Pain should be reassessed at regular intervals or when resident complains of persistent or worsening pain
- Reassess residents who are taking increasing doses of PRN medication and who are regularly scheduled for pain medication

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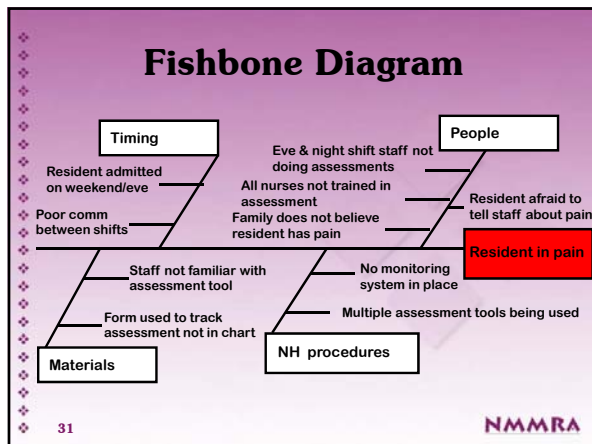
Understand Process Variation

Root Cause Analysis: A structured investigation that aims to identify the true cause of a problem and the actions necessary to eliminate it



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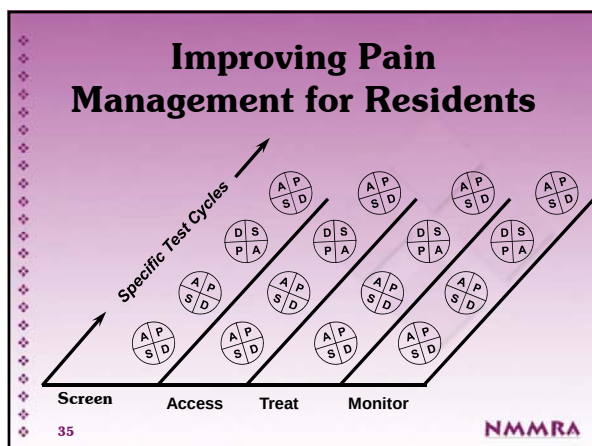
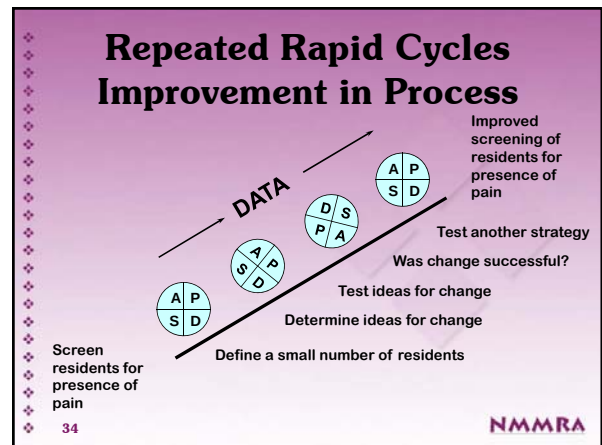
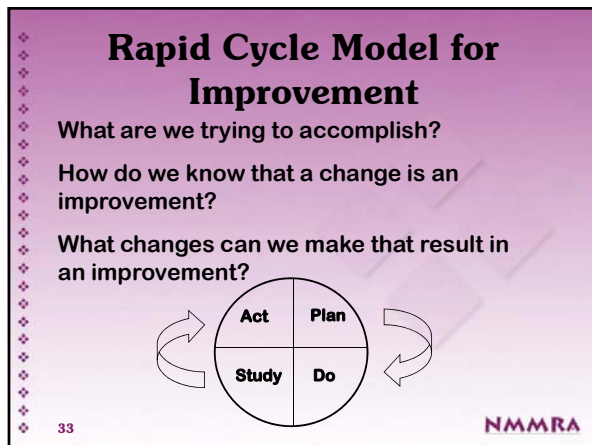
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Select the Process for Improvement

- What changes can we make that result in an improvement?
- Apply best practices or standards to organizational process

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Being Successful with Rapid Cycle Improvement

- Plan multiple cycles to test and adapt change
- Think a couple of cycles ahead
- Scale down size of test
- Collect useful data during each test
- Test over a wide range of conditions

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Fundamental Questions for Improvement

- What are we trying to accomplish?
- What changes can we make that will result in an improvement?
- How will we know that a change is an improvement?

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Effective Pain Management

- Screen residents routinely for pain
- Complete assessment for residents with pain
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